
SOAP Note 1 – Low-Level Condition (In-Person Visit)

CC: “My throat’s been a little sore.”

S:

Patient is a 24-year-old with mild sore throat for the past 2 days. No fever, no difficulty swallowing, and no cough. Reports drinking hot tea helps. No sick contacts.

O:

Afebrile, vitals stable

Throat mildly red, no tonsillar swelling or exudates

Lungs clear

No lymph node enlargement

A:

Likely viral pharyngitis (J02.9).

P:

Reassured patient that antibiotics aren’t needed

Encouraged fluids, throat lozenges, acetaminophen as needed

Return if symptoms worsen or fever develops

SOAP Note 2 - Moderate-Level Condition (In-Person Visit)

CC: "I'm getting winded just going up the stairs."

S:

62-year-old smoker reports progressive shortness of breath over the past week. No chest pain. Has occasional dry cough. No fever.

O:

HR 92, RR 20, SpO₂ 95% on room air

Wheezing noted bilaterally

Heart regular, no murmurs

No swelling in legs

A:

Mild-moderate COPD exacerbation (J44.1).

P:

Started albuterol inhaler, instructed on proper use

Short prednisone course prescribed

Strongly encouraged smoking cessation

Close follow-up in 1 week or sooner if worsening

SOAP Note 3 – High-Level Condition (In-Person Visit)

CC: “Severe chest pain!”

S:
58-year-old male developed sudden, crushing chest pain radiating to left arm about 30 minutes ago. Associated nausea and sweating.

O:
HR 110, BP 92/60, SpO₂ 89%

Patient appears diaphoretic and anxious

Lungs: mild crackles

ECG: ST elevation in inferior leads

A:
Acute STEMI (I21.3).

P:
Gave aspirin, nitroglycerin, and oxygen immediately

Called STEMI team, activated cath lab

Patient being transferred for urgent PCI

Admitted to CCU

SOAP Note 4 – Moderate Condition (Audio-Only Telemedicine Visit)

CC: “My sinuses have been killing me all week.”

S:
41-year-old reports 7 days of nasal congestion, sinus pressure, and headache. Thick yellow nasal drainage. No fever. No visual symptoms or breathing problems.
Patient states they only have access to phone call (no video).

O:
Unable to examine directly – history obtained verbally only.

A:
Acute bacterial sinusitis (J01.90).

P:

Prescribed amoxicillin-clavulanate × 7 days

Recommended saline rinses, hydration, and rest

Reviewed red-flag signs (eye swelling, persistent fever, worsening pain)

Asked patient to call back if not better in 3-5 days

SOAP Note 5 – Moderate Condition (Audio & Video Telemedicine Visit)

CC: “I’ve got this itchy rash after yard work.”

S:

36-year-old reports itchy red rash on both arms for 3 days. No fever, no breathing issues, no swelling. Thinks it started after clearing brush in the yard.

O:

Video exam shows linear, red, blister-like rash on both forearms, consistent with poison ivy contact dermatitis. No facial swelling or respiratory distress.

A:

Allergic contact dermatitis from poison ivy (L23.7).

P:

Prescribed triamcinolone cream for rash

Recommended cetirizine for itching

Advised washing all clothes/linens that may have plant oil

Follow up if spreading, infected, or not improving

SOAP Note - Moderate Condition with Obesity Documentation

CC: "Here for blood pressure check."

S:
52-year-old male presents for follow-up on hypertension. Reports occasional mild headaches but denies chest pain, shortness of breath, or vision changes. Admits to poor diet and minimal exercise.

O:

Vitals: BP 148/92, HR 84, SpO₂ 98%

Height: 5'9" (175 cm), Weight: 248 lbs (112 kg) → BMI: 36.6

BMI places patient in Class II Obesity (BMI 35.0-39.9)

Heart regular, lungs clear, no edema

A:

Hypertension, uncontrolled (I10) - moderate complexity

Class II Obesity (E66.9, Z68.36 for BMI documentation)

P:

Increased lisinopril to 20 mg daily

Counseling on weight loss, diet, and exercise

Referral to nutritionist for obesity management

Return in 4 weeks for blood pressure and weight check

CPT Code: 99214 - Established patient, moderate complexity, in-person visit

Obesity Level: Class II (BMI 35-39.9)